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EXECUTIVE ASSESSMENT

EXECUTIVE SUMMARY

Canadian healthcare organizations continue to invest in digital technologies to improve operational efficiency, patient experience, and administrative workflows. While electronic medical records (EMRs) are now widely adopted, organizations continue to face challenges in achieving consistent use of newly introduced digital workflows. As a result, the success of digital transformation increasingly depends not only on technology implementation, but also on how effectively digital workflows are integrated into day-to-day operations.

This portfolio presents a simulated operational assessment of a fictional Regional Outpatient Centre (ROC). This project focuses on one speciality clinic where an EMR-integrated digital patient intake workflow was introduced to streamline patient registration and reduce administrative burden. Three months after implementation, adoption has plateaued at 57%, with staff continuing to rely on hybrid workflows, including paper/manual documentation and duplicate data entry.

The objective is to assess barriers to adoption of the existing EMR-integrated digital patient intake workflow and identify practical opportunities to improve its use and operational effectiveness. The project assesses the current workflow, stakeholder needs, adoption barriers, and operational performance before piloting targeted workflow and staff-support improvements.

The assessment uses Canadian healthcare evidence and illustrative operational data to demonstrate project coordination, stakeholder engagement, operational analysis, performance monitoring, and evidence-based recommendations in a healthcare setting.

PROJECT CONTEXT

Attribute	Description
Organization type	Regional outpatient healthcare center
Location	Ontario, Canada
Services	Primary care and outpatient specialty services
Workforce	Physicians, nurses, allied health professionals, administrative staff, and IT support

INITIATIVE

As part of its digital transformation strategy, the simulated ROC introduced an EMR-integrated digital patient intake workflow to standardize patient registration, reduce administrative effort, and improve the accessibility of intake information across clinical and administrative workflows.

PROBLEM STATEMENT

In the simulated case, adoption of the digital patient intake workflow three months after implementation has plateaued at 57%. Despite the availability of the EMR-integrated workflow, staff continued to use hybrid intake practices, including manual documentation and duplicate data entry. These practices reduced the consistency of the digital workflow and limited the operational benefits expected from the initiative.

ASSESSMENT OBJECTIVE

Assess research-informed factors that may influence adoption of the EMR-integrated digital patient intake workflow and identify practical opportunities for improving sustained use and operational effectiveness.

KEY FINDINGS



Digital transformation success depends on adoption, not just technology

Canadian healthcare organizations have largely implemented digital systems such as EMRs. The remaining challenge is ensuring consistent utilization of digital workflows to realize operational benefits.



Adoption barriers are primarily organizational

Digital-health evidence identifies integration, usability, training, and workflow-related barriers that can limit effective use of digital health systems.



Hybrid workflows reduce operational efficiency

Continued reliance on paper documentation and duplicate data entry limits standardization, increases administrative effort, and reduces the expected value of digital transformation initiatives.



Operational performance should be monitored beyond implementation

Sustained adoption requires measurable KPIs that enable leadership to monitor progress, identify barriers, and support continuous improvement.

ASSESSMENT SCOPE

IN SCOPE

- Existing EMR-integrated digital patient intake workflow
- Current state workflow and adoption assessment
- Stakeholder and barrier assessment
- Targeted workflow and staff-support improvements
- Pilot monitoring and KPI evaluation
- Recommendations for sustained adoption

OUT OF SCOPE

- Software development or customization
- EMR implementation
- Technical infrastructure
- Software procurement
- Enterprise-wide rollout

STAKEHOLDER ANALYSIS

STAKEHOLDER	ROLE	KEY CONCERNS	ENGAGEMENT
Executive Leadership	Project sponsorship, strategic oversight, escalation	Project outcomes, resource needs, sustainability	High
Clinical Operations Lead	Operational decisions and workflow leadership	Workflow efficiency, adoption, operational disruption	High
Clinical / Administrative representatives	Frontline workflow input and validation	Ease of use, workflow fit, training/support	High
Digital Health / IT Support	Technical support and issue resolution	Workflow functionality, technical issues, support requirements	Medium
Patients	End users of digital intake	Ease of completion, accessibility, assistance needs	Medium

ASSESSMENT APPROACH

The assessment uses a combination of baseline operational data, current-state workflow review, stakeholder analysis, and external Canadian digital-health evidence to identify factors influencing digital intake adoption.

The assessment follows stages:-

1. Establish the baseline: Measure digital intake adoption, manual/duplicate re-entry data, patient-assistance, and staff confidence.
2. Assess adoption barriers: Review the current workflow and identify workflow, staff, patient-facing and organizational barriers.
3. Evaluate targeted improvements: Pilot selected workflow and staff-support actions and monitor changes using defined KPIs.
4. Inform recommendations: Compare pilot results with baseline performance and targets, assess remaining barriers, and identify appropriate next steps.

EVIDENCE BASE

The assessment framework and barrier categories are informed by Canadian healthcare and digital-health sources, including:

- Canada Health Infoway
- Canadian Medical Association
- Canadian Institute for Health Information
- Digital Health Canada
- Digital Health Interoperability Task Force

Note: All clinical operational data are illustrative and created for project demonstration purposes. No actual patient, staff, or clinic records are used. External sources inform the assessment framework and interpretation of digital health adoption challenges. They do not represent findings from the simulated clinic.